



Jorge Armando Diaz, M.D.
INTERNAL MEDICINE

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1/15/2024

Amit Patel, MD
115 MEDICAL DRIVE SUITE 202
Victoria, TX. 77901
Phone: (281)836-4690, Fax: (409)729-2449.

Dear Dr. Patel,

I would like to refer the following patient to your service:

Mr. Joe Garza
310 Louise St.
Edna, TX. 77957
Home Phone: (512)538-4322
DOB: 9/4/1944
SSN: 462-66-3491

Mr. Garza was seen in our office on 01/04/2024. Attached please find the patients current medical evaluation;

CC:

Mr. Garza is a 79 year old White male. This is a follow-up visit. He is here for annual wellness visit. History details were provided by the patient. The history appears to be reliable.

HPI:

Encounter for general adult medical examination without abnormal findings noted. A chest x-ray was done in 04/04/2019 and it was normal. His last ECG was in 12/2020 and was normal. He has never had a hearing test. He has never had a flexible sigmoidoscopy or colonoscopy. A pharmacological stress test has never been performed. A treadmill stress test has never been performed. He's had vision screening done <6 months ago and this was normal. He performs testicular self-exams occasionally. He is current with his pneumococcal, hepatitis B, influenza, Shingrix and COVID, Pevnar immunization. John Hopkins Fall Risk Assessment Depression Screening

Smoking status: Nonsmoker (never smoked) Mr. Garza is a very pleasant 79-year-old Hispanic-American male patient with past medical history of dysphagia, achalasia of cardia, chronic inactive gastritis, Vitamin B12, and Vitamin D deficiency, gastric hyperplastic polyp diagnosed 1/28/2021, and erectile dysfunction. He is here today for his annual wellness visit. It is important to mention that we have not seen this patient since 06/28/2022. The patient admits that he did not follow-up with the undersigned in the last 1 and a half years since he has been doing well and he did not have any reason to follow-up. Today, he presents in good spirits, cooperative, and not in distress. The main concern of this patient today is his chronic dysphagia secondary to his achalasia of cardia. The patient reports that he already received 2 botulinum toxin injection in lower esophageal sphincter in the past which helped improved his symptoms. The patient does show interest in doing botulinum toxin injection again for comfort. The patient reports today during his meals, he had to drink a lot of fluid to push the food that he consumed because if not he will choke up. He adds that there are nights that he even wakes up since he is choking and had to spit up some fluids. On another note, the patient also reports that he has low back pain which he is managing by being constantly mobile. He explains that his low back pain is only prominent when he sits and become idle for long period of time but will improve once he starts walking. He adds that occasionally take Ibuprofen but only for severe low back pain. On another note, the patient denies any recent ER visits or hospitalizations in the last year. He has not fallen or been into any kinds of accident or trauma in the last 12 months. He denies any problems with his gait. He does not use or assist himself during ambulation with cane or walker. The patient remains independent and is able to perform his activities of daily living such as bathing, toileting, personal hygiene, eating on his own, driving and doing home tasks. He does not have urinary incontinence or stool incontinence. The patients cognitive ability remains to be intact. The patient has been screened for depression and he was negative. The patient denies having any trouble sleeping, issues with his appetite, or feelings of stress. The patient only drinks alcohol on special occasions. He does not smoke but admits history of smoking in the past. He quit smoking when more than 50 years ago. Regarding his colonoscopy, the patient was scheduled to have a Colonoscopy on 01/28/2021, but due to patient vomiting after taking the prep, Dr. Carlos Chinae canceled the procedure and patient was told to reschedule but the patient was not able to follow-up on this. Regarding his vaccines, he already got the COVID 19 latest booster and Influenza vaccine 2 days ago. He is due for his Prevnar 20, RSV, and Shingrix vaccines.

Past Medical History:

Dysphagia,
Acute Cholecystitis 04/01/19,
Chronic Inactive gastritis, Intestinal Metaplasia, Gastric hyperplastic polp 01/28/2021 and
Achalasia of Cardia K22.0, esophageal stricture
Erectile Dysfunction: dx'd in 10/07/2016; ;
Obesity: Moderate (BMI 30-34.9);,
Vitamin B12 deficiency E53.9 dx'd in 8/24/2021; and
Vitamin D deficiency E55.9 dx'd in 10/24/2017; ;

ADVANCE DIRECTIVES: Living will and Do Not Resuscitate verbal

PREVENTIVE HEALTH MAINTENANCE

COLONOSCOPY: has never been done
ENDOSCOPY: EGD: Abnormal (01/28/2021)
GLAUCOMA SCREEN: Glaucoma Screen: OD 15 OS 12 (10/24/2017)
EYE EXAM: was last done 11/2021
INFLUENZA VACCINE: Influenza Vaccination: 01/02/2024 (01/02/2024)

PREVNAR 13 VACCINE: Pevnar 13 (PCV-13): 08/01/2013 (08/01/2013)
PNEUMOCOCCAL VACCINE: Pneumococcal vaccination: 07/07/2014 (07/07/2014)
TETANUS VACCINE: was last done 01/01/2012
SHINGLES VACCINE: has never been done
COVID 19 VACCINE: was last done 02/11/2021 The next one is due 03/16/2021, 01/02/2024
Pevnar 20 01/04/2024

Past Surgical History:

Positive for
Appendectomy: 1970;,
Cataract Removal: bilateral; and
Cholecystectomy: 04/2019; ;
Positive for
Cataract Removal;
Positive for
Esophagastroduodenoscopy 01/28/21 Botox injected in lower esophageal sphincter;

Medications:

Last Reviewed on 1/04/2024 01:45 PM by Ocampo, Trixia Mae
cholecalciferol (vitamin D3) 50,000 unit oral capsule [Take 1 tab by mouth weekly]
omeprazole 40 mg oral capsule, delayed release (enteric coated) [take 1 capsule (40 mg) by oral route 2
times per day before a meal as needed]

Allergies:

No Known Drug Allergies

Social History:

Occupation: Retired, Plumbing;
Marital Status: Married
Children: 1 child (age 54)
Alcohol: Drinks alcohol on a social basis only (1-2 drinks per week on average). Tobacco: Past
history of cigarette smoking; quit 1972.
NEGATIVE

Mental Health History:

NEGATIVE

Communicable Diseases History:

Reportable health conditions; NEGATIVE

Family History:

Father:
Positive for Cerebrovascular Accident;
; Positive for Sleep Apnea;
Mother:
Positive for Type 2 Diabetes;
; Positive for Kidney Disease;

ROS:

CONSTITUTIONAL: Positive for unintentional weight gain (5.4 pounds; since 06/28/2022).

EYES: Negative for blurred vision, eye pain, and photophobia.

E/N/T: Negative for hearing problems, E/N/T pain, congestion, rhinorrhea, epistaxis, hoarseness, and dental problems.

CARDIOVASCULAR: Negative for chest pain, palpitations, tachycardia, orthopnea, and edema.

RESPIRATORY: Negative for cough, dyspnea, and hemoptysis.

GASTROINTESTINAL: Positive for diarrhea.

GENTOURINARY: Negative for dysuria, genital lesions, hematuria, impotence, polyuria, and changes in urine stream.

MUSCULOSKELETAL: Negative for arthralgias, back pain, and myalgias.

INTEGUMENTARY: Negative for atypical moles, dry skin, pruritis, and rashes.

NEUROLOGICAL: Negative for dizziness, headaches, paresthesias, and weakness.

HEMATOLOGIC/LYMPHATIC: Negative for easy bruising, bleeding, and lymphadenopathy.

ENDOCRINE: Negative for hair loss, heat/cold intolerance, polydipsia, and polyphagia.

ALLERGIC/IMMUNOLOGIC: Negative for allergies, frequent illnesses, HIV exposure, and urticaria.

PSYCHIATRIC: Negative for anxiety, depression, and sleep disturbances.

Physical Examination:

Ht: 5 ft, 5 in; Wt: 189.4 lbs; BMI: 31.5BP: 120/80 mm Hg (left arm, sitting); P: 86 bpm (left arm (BP Cuff), sitting); sCr: 1.05 mg/dL; GFR: 54.05

GENERAL: well developed, well nourished, in no apparent distress; moderately obese; well groomed; no apparent distress

EYES: arcus senilis; bilateral surgical pupils.;

E/N/T: normal external ears and nose;; normal external auditory canals and tympanic membranes;

Hearing: grossly normal **Nose:** normal nasal mucosa, septum, turbinates, and sinuses; **Lips, Teeth and Gums:** normal; **Oropharynx:** normal mucosa, palate, and posterior pharynx;

RESPIRATORY: normal respiratory rate and pattern with no distress; percussion is normal without hyperresonance or dullness; lung fields normal to palpation; normal breath sounds with no rales, rhonchi, wheezes or rubs;

CARDIOVASCULAR: normal PMI placement; no thrills, heaves, or lifts; normal rate and rhythm without murmurs; normal S1 and S2 heart sounds with no S3, S4, rubs, or clicks; carotids: 2+ amplitude, no bruits; abdominal aorta appears to be of normal size and is without bruits; femoral pulses: 2+ amplitude, no bruits; 2+ pedal pulses; no edema or significant varicosities;

BREASTS: symmetric; no overlying skin changes; no tenderness, nodularity, or masses;

SKIN: skin lesion on the right side of his forehead (please see picture);; no skin thickening, induration, or subcutaneous nodules;

GASTROINTESTINAL: normal bowel sounds; no masses or tenderness; no organomegaly, rectal exam: deferred guaiac negative stool; surgical scar(s) present include open cholecystectomy scar and appendectomy scar;

PSYCHIATRIC: mental status: alert and oriented x 3; appropriate affect and demeanor; recent and remote memory are intact; good insight and judgement;

MUSCULOSKELETAL: digits/nails: no clubbing, cyanosis, or evidence of ischemia or infection; normal gait; grossly normal tone and muscle strength; full, painless range of motion of all major muscle groups and joints no laxity or subluxation of any joints; no masses, effusions, misalignment, crepitus, or tenderness in major joints;

Laboratories:

HgbA1c: 5.3 (11/05/2019), 5.4 (12/16/2020), 5.4 (%A1C) (08/23/2021),
Vitamin D, 25-OH, D3: 25.1 (11/05/2019), 57.9 (12/16/2020), 40 (ng/mL) (08/23/2021),
B12: 172 (11/05/2019), 159 (12/16/2020), 135 (pg/mL) (08/23/2021),
TSH: 0.84 (11/05/2019), 0.56 (12/16/2020), 0.49 (uIU/mL) (08/23/2021),
Albumin, Serum: 4.3 (04/01/2019), 4.45 (12/16/2020), 3.8 (g/dL) (08/23/2021),
Bilirubin, Total: 1.6 (04/01/2019), 1.18 (12/16/2020), 0.6 (mg/dL) (08/23/2021),
Alkaline Phosphatase, Serum: 61 (04/01/2019), 57.2 (12/16/2020), 56 (IU/L) (08/23/2021),
AST (SGOT): 23 (04/01/2019), 15.4 (12/16/2020), 13 (IU/L) (08/23/2021),
ALT (SGPT): 18 (04/01/2019), 8 (12/16/2020), 7 (IU/L) (08/23/2021),
Protein, Total: 8.2 (04/01/2019), 7.25 (12/16/2020), 6.2 (g/dL) (08/23/2021),
Total Cholesterol: 159 (11/05/2019), 154 (12/16/2020), 148 (mg/dL) (08/23/2021),
HDL: 33 (11/05/2019), 37 (12/16/2020), 29 (mg/dL) (08/23/2021),
Triglycerides: 125 (11/05/2019), 83 (12/16/2020), 168 (mg/dL) (08/23/2021),
LDL: 101 (11/05/2019), 100 (12/16/2020), 85 (mg/dL) (08/23/2021),
VLDL Cholesterol: 34 (mg/dL) (08/23/2021),
CHOL/HDLRATIO: 4.1 (12/16/2020), 5.1 (Ratio) (08/23/2021),
NON HDL CHOL: 116.4 (12/16/2020),
Na: 141 (11/05/2019), 142 (12/16/2020), 141 (mmol/L) (08/23/2021),
K+: 4.7 (11/05/2019), 3.8 (12/16/2020), 3.9 (mmol/L) (08/23/2021),
Cl (serum): 103 (11/05/2019), 101 (12/16/2020), 104 (mmol/L) (08/23/2021),
CO2: 33 (11/05/2019), 32 (12/16/2020), 31 (mmol/L) (08/23/2021),
BUN: 15.3 (12/16/2020), 12 (mg/dL) (08/23/2021),
Creatinine, Serum: 1.13 (11/05/2019), 1.03 (12/16/2020), 1.05 (mg/dL) (08/23/2021),
Glucose, Plasma: 73 (mg/dL) (08/23/2021),
Glucose, Serum: 84 (11/05/2019), 87 (12/16/2020),
Calcium, Total: 9.5 (11/05/2019), 9.4 (12/16/2020), 8.8 (mg/dL) (08/23/2021),
Osmo: 280 (mosm/kg) (08/23/2021),
Anion Gap: 13 (12/16/2020),
eGFR: 63.2 (11/05/2019), 70.2 (12/16/2020), 68.5 (ml/min/1.73m2) (08/23/2021),
WBC count: 5.02 (11/05/2019), 7.42 (12/16/2020), 4.8 (K/uL) (08/23/2021),
Hemoglobin: 16 (11/05/2019), 16.6 (12/16/2020), 15.3 (g/dL) (08/23/2021),
Hematocrit: 47 (11/05/2019), 49.8 (12/16/2020), 47.6 (%) (08/23/2021),
MCV: 91.6 (11/05/2019), 91.4 (12/16/2020), 94.4 (fL) (08/23/2021),
MCH: 31.2 (11/05/2019), 30.5 (12/16/2020), 30.3 (pg) (08/23/2021),
RDW: 11.8 (11/05/2019), 11.9 (12/16/2020), 12.8 (%) (08/23/2021),
Platelet Count: 192 (11/05/2019), 178 (12/16/2020), 189 (K/uL) (08/23/2021),
Neutrophils: 60.1 (11/05/2019), 68.2 (12/16/2020), 65.5 (%) (08/23/2021),
Lymphs: 23.3 (11/05/2019), 19.7 (12/16/2020), 22.7 (%) (08/23/2021),
Monocytes: 11 (11/05/2019), 8.6 (12/16/2020), 8.1 (%) (08/23/2021),
Eos: 4.6 (11/05/2019), 2.6 (12/16/2020), 3.0 (%) (08/23/2021),
Basos: 0.8 (11/05/2019), 0.8 (12/16/2020), 0.7 (%) (08/23/2021),

ASSESSMENT & PLAN:

Encounter for general adult medical examination without abnormal findings

Advanced care planning has been discussed, pt wishes to adopt DNR status, 30 minutes has been spent on discussing Advanced Care planning.

COUNSELING was provided today regarding the following topics: healthy eating habits, low cholesterol diet, low salt diet, regular exercise, testicular self-exam, use of seat belts, and fall prevention.

Orders:

99497Advance care planning first 30 mins1158FAdvance care planning discussion documented in the medical record (COA)1159FMedication list documented in medical record (COA)2010FVital signs (temperature, pulse, respiratory rate, blood pressure) documented/reviewed (CAP)1(EM)53074FMost recent systolic blood pressure < 130 mm Hg (DM) (HTN)3078FMost recent diastolic blood pressure <80 mm Hg (HTN)1 (DM)493000Electrocardiogram, routine with at least 12 leads; with interpretation and reportG0439Annual wellness visit, includes a PPPS, subsequent visitG8752Most recent systolic BP<140 (normal)G8482Influenza immunization was ordered or administeredG8754Diastolic BP <901160FRev of all meds by prescribing practitioner (Rxs. OTC, herbal therapies, and supps) doc'd in recordG8427List of current meds with dosages and verification w/ patient or rep documented by the providerPatient Education Handouts:

MedlinePlus document: Encounter for general adult medical examination without abnormal findingsAchalasia of cardia

REFERRALS: Referral initiated to a gastroenterologist (At this time, the patient agrees that his symptoms are recurring and will benefit with another consultation with his gastroenterologist, Dr. Carlos Chinae for further evaluation and treatment. During the office visit, the undersigned personally sent a message to Dr. Chinae to inquire if he can see the patient for an evaluation however, we have not received any response yet. At this time, we will deploy referral to Dr. Chinae for possible EGD and esophageal dilation and we will send all pertinent documents. The patient verbalized his understanding and agreement. We will continue to monitor the patient).

Orders:

99213-25Established patient outpatient visit, Low MDM and/or 20-29 minutesRFGASGastrologist ReferralPatient Education Handouts:

MedlinePlus document: Achalasia of cardiaDysphagia, unspecified

At this time, the patient agrees that his symptoms are recurring and will benefit with another consultation with his gastroenterologist, Dr. Carlos Chinae for further evaluation and treatment. During the office visit, the undersigned personally sent a message to Dr. Chinae to inquire if he can see the patient for an evaluation however, we have not received any response yet. At this time, we will deploy referral to Dr. Chinae and we will send all pertinent documents. The patient verbalized his understanding and agreement. We will continue to monitor the patient.

MEDICATIONS: (no change to current medication regimen) (see today's med list)

FOLLOW-UP: Schedule a follow-up visit in 3 months.

Patient Education Handouts:

MedlinePlus document: Dysphagia, unspecifiedVitamin D deficiency, unspecified

Still awaiting lab results before making any medication changes or suggestions. The patient will continue on Vitamin D3 50,000 IU by mouth weekly. Further recommendations will be made once we receive the results of the labs. We will continue to monitor the patient.

LABORATORY: Labs ordered include Vitamin D level.

MEDICATIONS: (no change to current medication regimen) (see today's med list)

FOLLOW-UP: Schedule a follow-up visit in 3 months.

Patient Education Handouts:

MedlinePlus document: Vitamin D deficiency, unspecifiedVitamin B deficiency, unspecified

Still awaiting lab results before making any medication changes or suggestions. Further recommendations will be made once we receive the results of the labs. We will continue to monitor the patient.

MEDICATIONS: (no change to current medication regimen) (see today's med list)

FOLLOW-UP: Schedule a follow-up visit in 3 months.

Patient Education Handouts:

MedlinePlus document: Vitamin B deficiency, unspecifiedEncounter for immunization

Patient received Prevnar 20 vaccination without any complications. We will defer administration of Shingrix at this time since he just received COVID 19 2 days to prevent possible side effects.

RECOMMENDATIONS given include: Mr. Garza was advised to get the following immunizations:

Pneumonia vaccine Prevnar in office today.

IMMUNIZATIONS given today: Prevnar 20.

Orders:

G0009Medicare administration of pneumonia vaccineImmunizations:

90677PCV20 VACCINE IM0.5left armintramuscularYap-Diaz, Lorraine

Pfizer, IncHe617503/01/2025

00005-2000-10

Patient Education Handouts:

MedlinePlus document: Encounter for immunizationEncounter for screening for malignant neoplasm of colon

REFERRALS: Referral initiated to a gastroenterologist (Dr. Carlos Chinae; The patient will be referred again to Dr. Carlos Chinae for colonoscopy procedure. The patient has been re-assured that the laxative solution that he used in the past which has caused him nausea and vomiting is already obsolete and that we have a new colonoscopy preparation solution called SUPREP for comfort and compliance that we will recommend Dr. Chinae to use for him instead. The patient verbalized his understanding and agreement. We will deploy referral today.).

Orders:

3017FColorectal CA screen results documented and reviewed (PV)Patient Education Handouts:

MedlinePlus document: Encounter for screening for malignant neoplasm of colonEncounter for screening for malignant neoplasm of prostate

As of May/08/2018 The USPSTF recommends against PSA-based screening for prostate cancer in men 70 years and older.

RECOMMENDATIONS given include: limit beverages in the evening, limit caffeine, decrease alcohol consumption, don't hold the urine too long, avoid anticholinergics, antihistamines or diuretics, and No PSA is needed based on Patients age and mutiple comorbidities..

FOLLOW-UP: Repeat screening in one year.Repeat screening in one year.

Patient Education Handouts:

MedlinePlus document: Encounter for screening for malignant neoplasm of prostateEncounter for screening for malignant neoplasm of skin

The patient will be referred to dermatologist, Dr. Amit Patel, to evaluate the skin lesion on the patient's right temple (please see picture) to rule out skin cancer. The patient verbalized his understanding and agreement. We will continue to monitor the patient.

RECOMMENDATIONS given include: use sunscreen at all times when exposed to sunny areas..

Repeat screening in 1 year;

FOLLOW-UP: Schedule a follow-up visit in 3 months.

Orders:

2029FComplete physical skin exam performed (ML)Patient Education Handouts:

MedlinePlus document: Encounter for screening for malignant neoplasm of skinEncounter for screening examination for other mental health and behavioral disorders

Patient was given the mini mental examination and clock drawing test in office. Results were negative.

MEDICATIONS: (no change to current medication regimen) (see today's med list)

RECOMMENDATIONS given include: The patient was advised to keep advanced directives in check, assigned a proxy and healthcare power of attorney, make financial arrangements in the event that patient would suffer inability to make her own financial decisions, that is to assign a power of attorney for financial needs. and Follow up in 1 year.

Repeat screening in 1 year;

Orders:

2014FMental status assessed (CAP)1(EM)53755FCognitive+behavioral impairment scrng performedPatient Education Handouts:

MedlinePlus document: Encounter for screening examination for other mental health and behavioral disordersEncounter for screening for depression

Patient was screened in office using PHQ9 and results were negative. Total time spent discussing: 10 mins

MEDICATIONS: (no change to current medication regimen) (see today's med list)

Repeat screening in 1 year;

FOLLOW-UP: Schedule a follow-up visit in 3 months.

Orders:

G0444-59Depression ScreeningG8510NEGATIVE DEPRESSIONPatient Education Handouts:

MedlinePlus document: Encounter for screening for depressionEncounter for screening for other disorder

Patient was screened using Alcohol Use Disorders Identification Test (AUDIT) Questionnaire in office today and results were negative. Total time spent discussion: 10mins.

MEDICATIONS: (no change to current medication regimen) (see today's med list)

Repeat screening in 1 year;

FOLLOW-UP: Schedule a follow-up visit in 3 months.

Orders:

3016FPatient screened for unhealthy alcohol use using a systematic screening method (pv)G0442-59Alcohol Behavior ScreeningPatient Education Handouts:

MedlinePlus document: Encounter for screening for other disorderEncounter for screening for other disorder

Patient was screened for Sleep apnea using the Stop Bang Questionnaire with a score of 3. However the patient does not reunites symptoms of sleep apnea such as daytime fatigued, or daytime sleepiness, therefore we will not send the patient for a sleep study.

MEDICATIONS: (no change to current medication regimen) (see today's med list)

Repeat screening in 1 year;

FOLLOW-UP: Schedule a follow-up visit in 3 months.

Patient Education Handouts:

MedlinePlus document: Encounter for screening for other disorder
Encounter for screening for other disorder

KATZ screening was performed in office. Patient scored 6 out of 6 and is independent.

MEDICATIONS: (no change to current medication regimen) (see today's med list)

Repeat screening in 1 year;

Orders:

1090FPresence or absence of urinary incontinence assessed (GER)1170FFunctional status assessed (COA) (RA)Patient Education Handouts:

MedlinePlus document: Encounter for screening for other disorder
Encounter for screening for other disorder

Currently, patient is at low risk for falls. He has a score of 0 on John Hopkins Fall Risk Assessment. No treatment is needed at this time.

MEDICATIONS: (no change to current medication regimen) (see today's med list)

Repeat screening in 1 year;

FOLLOW-UP: Schedule a follow-up visit in 3 months.

Orders:

0518FFalls plan of care documented1101FPt screen for fall risk; document no falls in past year or only 1 fall w/o injury in past year (GER)3288FFalls risk assessment documentedPatient Education Handouts:

MedlinePlus document: Encounter for screening for other disorder
Encounter for screening for other disorder

Patient was screened for tobacco use. Patient is a non tobacco user and does not have a prior history of smoking.

Repeat screening in 1 year;

Orders:

1000FTobacco use assessed (CAD, CAP, COPD, PV)1 (DM)41036FCurrent tobacco non-user (CAD, CAP, COPD, PV)1 (DM)4G9903Non SmokerPatient Education Handouts:

MedlinePlus document: Encounter for screening for other disorder
Body mass index [BMI] 31.0-31.9, adult

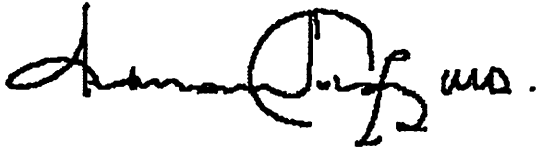
Patient was advised to research sugar addiction and carbohydrate addiction. We informed the patient to download mapmywalk app on his phone and walk 1 mile daily. We have explained to patient that weight loss comes up often in nutrition and fitness. Good weight loss programs combine daily exercise and healthy eating choices. The individual desiring to lose weight will need to restrict total caloric intake (no more than 1,000 Calories per day lower than normally required). Safe weight loss programs never go below 1,200 Calories per day. Safe and effective weight loss should not exceed two pounds per week. Although this may seem slow to some, it is necessary to avoid creating a yo-yo effect as the body attempts to return to its original weight. Slow weight loss with regular exercise helps the body adjust to the new weight more readily. Some general recommendations include the following: Mix protein (fish, egg whites, skinless chicken, skinless turkey, extra lean meats, lean lunch meats, legumes, soybean, dairy, low-fat cheeses, etc.) with carbohydrates every time you eat. Choose fruit or other foods low in fat with a relatively low glycemic index instead of giving in to the junk food craving. Waiting 20 minutes after making a healthier choice often makes the craving go away. Small healthy snacks can help reduce hunger and maintain a higher metabolism. Increase fiber intake (25 to 35 grams per day). Eat your larger meals earlier in the day when the metabolism is highest. This also increases the efficiency of your fat burning throughout the day. Breakfast and lunch should be the largest meals. Include strength training at least two days per week. Include over 20 minutes of cardiovascular training most days of the week

Orders:

2001FWeight recorded (CHF, PAG)3008FBody Mass Index (BMI), documented (PV)G8417>30, Diet & Exercise counseling

We look forward to working collaboratively in the benefit of our common patient. Please do not hesitate to call for any assistance I may be to Mr. Garza.

Sincerely,

A handwritten signature in black ink, appearing to read "Armando Diaz MD." with a stylized flourish at the end.

J. Armando Diaz, MD

Jorge Armando Diaz, M.D.
2710 Hospital Dr. Suite 306
Victoria, TX 77901-5748

Chart Cover

Print Date : 01/15/2024
Print Time : 12:42 pm
Print User: Pinili, Mariah

Garza, Joe H
310 Louise St.
Edna, TX 77957

Account No GARJOE0001
Home Phone (512)538-4322
Cell Phone (512)538-4322
Home Fax
Pager
Financial Group MDC

Date of Birth 09/04/1944
SSN XXX-XX-3491
Gender M
DL #
Marital Status M
First Visit Date 10/07/2016

Provider Diaz, Jorge Armando
Referral N/A

Employment Information

Patient Position
e-mail

Office Phone
Office Fax

Guarantor Information

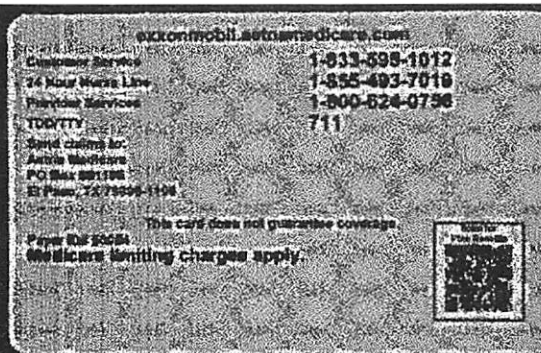
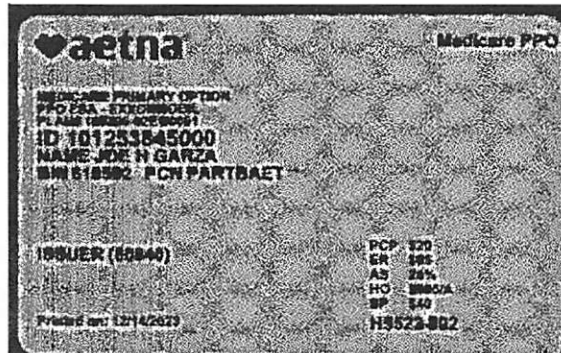
Garza, Joe H
310 Louise St.
Edna, TX 77957

Account No GARJOE0001
Home Phone (512)538-4322
Home Fax
e-mail

Gender M
Date of Birth 09/04/1944
DL #
SSN XXX-XX-3491

Insurance Information

Insurance Company Address	Office Phone City	Code State	Policy Holder Zip	Policy Number Group Number	Copayment	Deductible	% Ins
Aetna-Texas Claims Medicare P O Box 981106	(888)342-3862 El Paso	C1 TX	Garza, Joe H 79998-1106	101253845000 100296-02EG0001	\$20.00	\$0.00	100%



Health Summary

Notes